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Multi Superspeciality Hospital & Research Centre

Healing by Heart

2D Echocardiography & Colour Doppler Study

Name : Saroj Agarwal
Ref.by : Dr. Sanjay Agrawal

Age/Sex : 63 Years/Female
Date : 06/07/2026

OBSERVATION :

LV Mild Concentric LVH
No Significant RWMA noted at rest, Jerky IVS
Fair LV systolic function ; LVEF~ 50 %
Diastolic Dysfunction Grade I

Mitral valve: No MS/Mild MR
Aortic valve: No AS/Trivial AR
Tricuspid valve & Pulmonary valve normal
Mild to Moderate TR, Severe PAH (RVSP 70 mmHg.)
Normal LA, Mildly Dilated RA & RV; Normal RV function
IVC Normal (Size 12 mm), good collapsing
Intact IAS/IVS
Aorta normal root & arch
No intracardiac clot / vegetation
No Pericardial effusion

CONCLUSION : C/o Endometrial Carcinoma & on Chemotherapy
H/O Pulmonary Embolism, S/P Mechanical Thrombectomy (03/2026)

No significant RWMA noted at rest
Fair LV systolic function, LVEF: 50%
Mild Concentric LVH
Diastolic Dysfunction Grade I
Mild MR, Mild to Moderate TR
Severe PAH (PASP: 70 mmhg)
Mildly dilated RA & RV
IVC normal (Size 12 mm), good collapsing
No Vegetation/clot/or effusion

Dr. Narendra Tanwar
DM Cardiologist

Note : Normal Echo does not rule out coronary artery disease



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Univesity Road, Surat - 395007. / Ph. : 0261-2299000 / Reception :- +91 82382 29900



मैत्रेय
Multi Superspeciality Hospital & Research Centre
Healing by Heart

Name : SAROJ - AGARWAL
Age/Sex : 63 Yrs./Female
Patient Id : 59568/SD
Treating Dr. : Dr. SANJAY AGRAWAL

Date : 7-Jul-2026 6:25 am
Rec. No : 5847
Indoor No. : I/0726/907
Ref. By Dr. : DR. SANJAY AGRAWAL

BIOCHEMISTRY

TEST	RESULTS	Unit	REFERENCE RANGE
 Creatinine Method : Enzymatic	: 1.20	mg/dl	0.5 - 1.1 mg/dl
Electrolytes			
 Sodium Method : Direct ISE	: 135	mEq/L	135 - 155 mEq/L
 Potassium Method : Direct ISE	: 3.89	mEq/L	3.5 - 5.5 mEq/L
 Chloride Method : Direct ISE	: 98	mEq/l	98 - 107 mEq/l

Specimen : Serum

TEST DONE BY EM200 FULLY AUTOMATED BIOCHEMISTRY / EASYLYTE PLUS ELECTROLYTE ANALYZER

Verified By

Dr. Jitendra Kumar V. Dubey MBBS,MD
(Pathology)
Consultant Pathologist (GN No: 15107)

Operator Name : Wilson Valvi

Print Date & Time : 07-Jul-2026 07:01:00AM

Emergency 24 x 7 & 365 Days.

Corroborative Clinicopathologic Correlation is Essential for Final diagnosis. Test reports are subject to technical limitation & should be Clinically Co-related. Laboratory may be Contacted if Required.

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Multi Superspeciality Hospital & Research Centre
Healing by Heart

Name : SAROJ - AGARWAL
Age/Sex : 63 Yrs. / Female
Patient ID : 59568/IC
Treating Dr. : Dr. SANJAY AGRAWAL

Date : 06-Jul-2026 6:10 am
Receipt No : 5753
Indoor No : I/0726/907
Ref. By Dr. : DR. SANJAY AGRAWAL

ROUTINE URINE EXAMINATION REPORT

Test Name	Results	Units	Bio. Ref. Interval
PHYSICAL EXAMINATION :			
Quantity	: 15 ml		
Colour	: Pale Yellow		
Clarity	: Clear		
CHEMICAL EXAMINATION :			
pH (pH Indicator)	: 6.0		4.6 - 8.0
Specific Gravity (Ionic Concentration)	: 1.010		1.002 - 1.030
Protein (Tetrabromophenol Blue)	: Absent		Absent
Glucose (By GOD POD)	: Absent		Absent
Ketones (Sodium Nitroprusside)	: Absent		Absent
Bilirubin (Diachloroanilinediazonium)	: Absent		Nil
Nitrite (Tetrahydrobenzoquinolinol)	: Absent		Nil
Leucocytes (By Leuco. esterase)	: Normal		Nil
Blood (Tetramethylbenzidine)	: Negative		Absent
MICROSCOPIC EXAMINATION :			
Leucocytes(Pus Cells)	: Occasional/hpf.		0-5/hpf
Erythrocytes (RBC)	: Absent		Nil
Epithelial Cells	: 2-3/hpf.		0-5/hpf
Bacteria	: Absent		Absent
Casts	: Absent		Absent
Crystals	: Absent		Absent
Yeast cell	: Absent		Absent
Mucus	: Absent		Absent
Other Findings	: NIL		Nil

Sample Type : Urine

Verified By

Dr. Jitendrakumar Dubey MBBS,MD (Pathology)
Consultant Pathologist (GRN No. 15107)

Operator Name: Amrita PrintDate & Time: 06-Jul-2026 12:36:00PM Emergency 24 Hours 365 Days

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Name : SAROJ - AGARWAL
 Age/Sex : 63 Yrs. Female
 Patient ID : 59568/IC
 Treating Dr. : Dr. SANJAY AGRAWAL

Date : 05-Jul-2026 9:48 pm
 Receipt No : 5744
 Indoor No : I/0726/907
 Ref. By Dr. : DR. SANJAY AGRAWAL

Complete Blood Count

PARAMETER	RESULT	UNIT	BIOLOGICAL REFERENCE RANGE
Hemoglobin (Cyanide free reagent colorimetry)	11.4	gm %	11-15
Total RBC Count (Electrical Impedance)	3.57	m/c.mm	3.9-5.6
Total Leucocytes count(Electrical Impedance)	7270	/c.mm	4,000 - 10,000
Platelet Count (Electrical Impedance)	187000	/c.mm	150000 - 450000
Hematocrit (PCV) (Calculated)	33.7	%	36-47
MCV (Calculated)	94.4	fl	75-96
MCH (Calculated)	31.9	pg	27-32
MCHC (Calculated)	33.8	g/dl	30-36
RDW - CV (Calculated)	15.6	%	11- 16
Differential WBC Count (Microscopy)			
Band Form	00	%	0-5
Neutrophils	70	%	40-75
Lymphocytes	16	%	20-50
Eosinophils	05	%	1-6
Monocytes	09	%	2-10
Basophils	00	%	0-5
Neutrophil/Lymphocyte Ratio (Calculated)	4.38	Ratio	1.1-3.5
Absolute Count			
Neutrophils	5089	/cumm	2000-7000
Lymphocytes	1163	/cumm	1000-3000
Eosinophils	364	/cumm	20-500
Monocytes	654	/cumm	200-1000
Peripheral Smear Examination (Microscopy)			
Platelets On Smear	Adequate On Smear		
Malarial Parasites	Malaria Parasites Not Detected		
Sample Type : EDTA WHOLE BLOOD			

TEST DONE BY 5 PART FULLY AUTOMATIC H560 HEMATOLOGY ANALYZER

Verified By

(Signature)

Dr. Jitendra Kumar V. Dubey MBBS, MD

Consultant Pathologist (GRN No. 15107)

Operator Name : Wilson Valvi

Emergency 24 x 7 & 365 Days. Print Date & Time: 05-Jul-2026 10:40:00PM

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Name : SAROJ - AGARWAL
Age/Sex : 63 Yrs. Female
Patient ID : 59568/IC
Treating Dr. : Dr. SANJAY AGRAWAL

Date : 05-Jul-2026 9:48 pm
Receipt No : 5744
Indoor No : I/0726/907
Ref. By Dr. : DR. SANJAY AGRAWAL

Erythrocyte Sedimentation Rate

Test Name	Results	Units	Bio. Ref. Interval
Erythrocyte Sedimentation Rate	18	mm/hr	17-50 Yr : <10 51-60 Yr : <12 61-70 Yr : <14 >70 Yr : <30

Method : Westergren Method

Sample Type: EDTA Whole Blood

Note:

ESR is increased in all conditions where there is tissue breakdown or where there is entry of foreign proteins in the blood, except for localized mild infections. The determination is useful to check the progress of the disease. The changes of ESR are, however, not diagnostic of any specific disease.

Verified By

Operator Name : Amrita Emergency 24 x 7 & 365 Days. Print Date & Time :

Dr. Jitendrakumar Dubey MBBS, MSc
(Pathology)
Consultant Pathologist (GRN No: 15104)

06-Jul-2026 12:59:00 PM

Corroborative Clinicopathologic Correlation is Essential for Final diagnosis. Test reports are subject to technical limitation & should be Clinically Co-related. Laboratory may be Contacted if Required. Emergency 24 Hours.

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Name : SAROJ - AGARWAL
Age/Sex : 63 Yrs./Female
Patient Id : 59568/IC
Treating Dr. : Dr. SANJAY AGRAWAL

Date : 5-Jul-2026 9:48 pm
Rec. No : 5744
Indoor No. : I/0726/907
Ref. By Dr. : DR. SANJAY AGRAWAL

BIOCHEMISTRY

TEST	RESULTS	Unit	REFERENCE RANGE
 GGT Method : Glupa - c Method	: 32.9	U/L	0 - 38 U/L
 SGOT Method : IFCC Without P5P	: 16	U/L	0 - 31 U/L
 SGPT Method : IFCC Without P5P	: 14	U/L	0 - 34 U/L
 Alkaline Phosphatase Method : PNPP, AMP Buffer Method	: 69	IU/l	35 - 105 IU/l
 Total Bilirubin Method : Diazo Method	: 0.22	mg/dl	0.1 - 1.2 mg/dl
 Direct Bilirubin Method : Diazo Method	: 0.10	mg/dl	0.1 - 0.4 mg/dl
 Indirect Bilirubin Method : Calculated	: 0.12	mg/dl	0 - 0.75 mg/dl
 Creatinine Method : Enzymatic	: 1.08	mg/dl	0.5 - 1.1 mg/dl
 S. Protein Method : Biuret Method	: 5.63	gm/dl	6 - 8 gm/dl
 S.Albumin Method : Bromocresol Green (BCG) Method	: 3.28	g/dl	3.2 - 4.6 g/dl
 Globulin Method : Calculated	: 2.35	gm/dl	2 - 3.5 gm/dl
 A/G Ratio Method : Calculated	: 1.4	gm/dl	1.1 - 2.2 gm/dl
<u>Electrolytes</u>			
 Sodium	: 136	mEq/L	135 - 155 mEq/L

Verified By

Dr. Jitendra Kumar V. Dube MBBS, MD
(Pathology)
Consultant Pathologist (GRN No: 15107)

Operator Name : Wilson Valvi Print Date & Time : 05-Jul-2026 10:41:00PM Emergency 24 x 7 & 365 Days.
Corroborative Clinicopathologic Correlation is Essential for Final diagnosis. Test reports are subject to technical limitation & should be Clinically Co-related. Laboratory may be Contacted if Required.

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Name : SAROJ - AGARWAL

Age/Sex : 63 Yrs./Female

Patient Id : 59568/IC

Treating Dr. : Dr. SANJAY AGRAWAL

Date : 5-Jul-2026 9:48 pm

Rec. No : 5744

Indoor No. : I/0726/907

Ref. By Dr. : DR. SANJAY AGRAWAL

BIOCHEMISTRY

TEST	RESULTS	Unit	REFERENCE RANGE
Method : Direct ISE			
 Potassium	: 3.57	mEq/L	3.5 - 5.5 mEq/L
Method : Direct ISE			
 Chloride	: 98	mEq/l	98 - 107 mEq/l
Method : Direct ISE			

Specimen : Serum

TEST DONE BY EM200 FULLY AUTOMATED BIOCHEMISTRY / EASYLYTE PLUS ELECTROLYTE ANALYZER

Verified By


Dr. Jitendrakumar V. Dey MBBS, MD
(Pathology)
Consultant Pathologist (GRN No: 15107)

Operator Name : Wilson Valvi

Print Date & Time : 05-Jul-2026 10:41:00PM

Emergency 24 x 7 & 365 Days.

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Name : SAROJ - AGARWAL
Age/Sex : 63 Yrs. Female
Patient Id : 59568/IC
Treating Dr. : Dr. SANJAY AGRAWAL

Date : 05-Jul-2026 9:48 pm
Rec. No : 5744
Indoor No : I/0726/907
Ref. By Dr. : DR. SANJAY AGRAWAL

CRP(C- Reactive Protein)

TEST	RESULTS	UNIT	REFERENCE RANGE
REPORT OF C-REACTIVE PROTEIN			
C-Reactive Protein (CRP)	9.0	mg/L	0-6

Method: - Turbidimetricimmuno Assay (QUANTITATIVE)

Interpretation of Result.

CRP is an acute phase reactant or a substance within the bloodstream that is increased during an inflammation process. Any inflammation in the body can cause rise in CRP. Other causes of inflammation in the body are smoking. Weighing too much and even low - grade infections from bacteria and viruses. Increases in the CRP should not be interpreted without a complete clinical history. Measurement should be compared to previous values. Recent medical events resulting in tissue injury infection or inflammation which may cause elevated CRP level should also be considered when interpreting the result.

Verified By

Dr. Jitendra Kumar V. Dubey MBBS,MD
(Pathology)
Consultant Pathologist, GRN No: 15107)

Operator Name : Wilson Valvi

Emergency 24 x 7 & 365 Days.

Print Date & Time : 05-Jul-2026 10:40:52PM

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Name : SAROJ - AGARWAL
Age/Sex : 63 Yrs. Female
Patient ID : 59568/IC
Treating Dr. : Dr. SANJAY AGRAWAL

Date : 05-Jul-2026 9:48 pm
Receipt No : 5744
Indoor No : I/0726/907
Ref. By Dr. : DR. SANJAY AGRAWAL

NT PRO BNP

Investigation	Observed Value	Unit	Biological Reference Interval
Type Of Sample : Blood			
NT-Pro-BNP*	151.40	pg/mL	Refer Interpretation

Introduction :

NT-proBNP = N (or Amino)terminal pro Brain Natriuretic peptide.

It belongs to the family of Natriuretic Peptide (NP) such as BNP is produced by cardiac cell in response to the stretch of a ventricle due to overload (Hemodynamic Stress). Increased hemodynamic stress such as in Heart failure, leads to increased release of NT-proBNP in the circulation.

Interpretation :

Primary care patients - suspicion of chronic heart failure

<75 years : < 125 pg/ml HF Unlikely

>125 pg/ml HF Likely

>75 years : < 450 pg/ml HF Unlikely

> 450 pg/ml HF Likely

emergency department patients - patient presenting with acute dyspnea

Whatever the Age : <300 pg/ml HF Unlikely

< 50 years : > 450 pg/ml HF Likely

Reference :

1) Value of N terminal proBNP in the diagnosis of left ventricular systolic dysfunction in primary patient referred for echocardiography gustafsson et al. Heart drug 2003;3-141-146.

2) ICON Study - james L. Januzzi European Heart journal (2006) 27,330-337

Verified By

Dr. Jitendrakumar Dubey MBBS, MD
(Pathology) SURAT
Consultant Pathologist (GRN No: 15109)

Operator Name : Wilson Valvi Emergency 24 x 7 & 365 Days. Print Date & Time :

05-Jul-2026 9:48 pm

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Name : SAROJ - AGARWAL
Age/Sex : 63 Yrs. Female
Patient Id : 59568/SD
Treating Dr. : Dr. SANJAY AGRAWAL

Date : 06-Jul-2026 1:31 pm
Rec. No : 5796
Indoor No : I/0726/907
Ref. By Dr. : DR. SANJAY AGRAWAL

Iron Profile

TEST	RESULTS	UNIT	REFERENCE RANGE
Iron	55.7	µg/dL	Male : 59 - 158 Female : 37 - 145
TOTAL IRON BINDING CAPACITY (TIBC)	310	µg/dl	Male: 250 - 450 µg/dl Female: 250 - 450 µg/dl
% TRANSFERRIN SATURATION	17.96	%	13 - 45 %

INTERPRETATION : Iron determinands are performed for the diagnosis & monitoring of treatment of all the microcytic hypochromic diseases such as Iron deficiency anemia, hemochromatosis, chronic renal diseases as well as macrocytic anemia & also normocytic anemia, hemolytic anemia, hemoglobinopathies, bone marrow diseases & toxic bone marrow damage interpretation of iron status must be correlated with other parameters given below as a whole studies rather than interpreting a single test.

1. IRON DEFICIENCY ANEMIA: S.Iron (Low), TIBC(High), Transferrin saturation(low), Ferritin(low), HB Electrophoresis (Normal), Smear(Micro/hypo)
2. ANEMIA OF CHRONIC DISEASES: S.Iron (Low), TIBC(low), Transferrin saturation(low), Ferritin(Normal/High), HB Electrophoresis (Normal), Smear(Micro/hypo/Normal)
3. THALASSEMIA : S.Iron (Normal/High), TIBC(Normal), Transferrin saturation(Normal/High), Ferritin(High), HB Electrophoresis Abnormal, Smear(Micro/hypo with target cells)
4. SIDEROBLASTIC ANEMIA : S.Iron (Normal/High), TIBC(Normal/High), Transferrin saturation(Normal/High), Ferritin(High), HB Electrophoresis Normal, Smear (Micro/hypo/Variable)
5. IRON OVERLOAD (HEMOCHROMATOSIS): S.Iron (High), TIBC(low), Transferrin saturation(High), Ferritin(High), HB Electrophoresis Normal, Smear(Variable)
6. MEGALOBlastic ANEMIA : S.Iron (High), TIBC(low), Transferrin saturation(High), Ferritin(High), HB Electrophoresis Normal, Smear(Macrocytic) (Harrison's principles of internal medicine vol - 1, 15th edition/663)

Verified By

Operator Name : Amrita

Emergency 24 x 7 & 365 Days.

Print Date & Time : 06-Jul-2026 02:28:57PM

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Dr. Jitendra Kumar V Dubey MBBS MD
(Pathology)
Consultant Pathologist (GRN No. 15107)



Name : SAROJ - AGARWAL
Age/Sex : 63 Yrs. Female
Patient Id : 59568/SD
Treating Dr. : Dr. SANJAY AGRAWAL

Date : 06-Jul-2026 1:31 pm
Rec. No : 5796
Indoor No : I/0726/907
Ref. By Dr. : DR. SANJAY AGRAWAL

FERRITIN ESTIMATION

TEST	RESULTS	UNIT	REFERENCE RANGE
FERRITIN LEVEL			
FERRITIN, Serum	<u>60.7</u>	µg/L	MALE(>5 yrs.): 15-200 FEMALE(>5 yrs.): 15-150 Children(<5 Years) : 12-200

Note: Increase in serum ferritin due to inflammatory conditions (Acute phase response) can mask a diagnostically low result.

Introduction : Ferritin is a high-molecular weight iron-containing protein that functions in the body as an iron storage compound. It has been demonstrated that the Ferritin molecule, when fully saturated, may consist of over 20% iron by weight. Approximately 25% of the iron in a normal adult is present in various storage forms. About two-thirds of the iron stores in the human body exist in the form of Ferritin.

Use : Diagnosis of iron deficiency or excess; correlates with total body iron stores

?Predict and monitor iron deficiency?Determine response to iron therapy or compliance with treatment.

?Differentiate iron deficiency from chronic disease as cause of anaemia.?Monitor iron status in patients with chronic renal disease with or without dialysis.

?Detect iron overload states and monitor rate of iron accumulation and response to iron depletion therapy.?Population studies of iron levels and response to iron supplement.

Clinical Significance :

Decreased in: Iron Deficiency anaemia

Increased in: Ferritin is an acute-phase increased in many patients with various acute and chronic liver diseases, alcoholism, infection and inflammation, hyperthyroidism, Gaucher disease, acute myocardial infarction, iron overload, Renal cell carcinoma, Serum Ferritin may not be decreased when iron deficiency coexists with these conditions. Variations due to age Increases: with age, higher in men than women, in women on OCP, in person who eat red meat compared to vegetarians DILUTION PROTOCOL: (on request) At our lab with kit, manual dilution protocol has been validated for FERRITIN up to 1:20 dilution and result up to 40000 NG/ML. After above dilution, it will be done manually and because of Ag-Ab reaction curve it may be erroneous if diluted after validated dilution.

Adv.: Kindly Correlate Clinically

Verified By

Dr. Jitendrakumar V. Dubey MBBS MD
(Pathology)

Consultant Pathologist (GRN No. 35107)

Operator Name : Amrita

Emergency 24 x 7 & 365 Days.

Print Date & Time : 06-Jul-2026 02:29:19PM

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ID: 5114

06-07-2026 08:59:59

SAROJ - AGARWAL

UHID: 59568 Age: 63 Yrs./ema

Indoor No: 1/0726/907

DOA: 05-Jul-2026 9:22 pm

DR. SANJAY AGRAWAL

REF.: DR. SANJAY AGRAWAL

Diagnosis Inform

Sinus Rhythm

Largd PtfV1

Incomplete Right Bundle Branch Block

Abnormal Q Wave(III)

Right Ventricular Hypertrophy

Subsequent T Wave Abnormality

Prolonged QT Interval

Indeterminate Axis

Report Confirmed by:

HR 1 bpm

P 06 ms

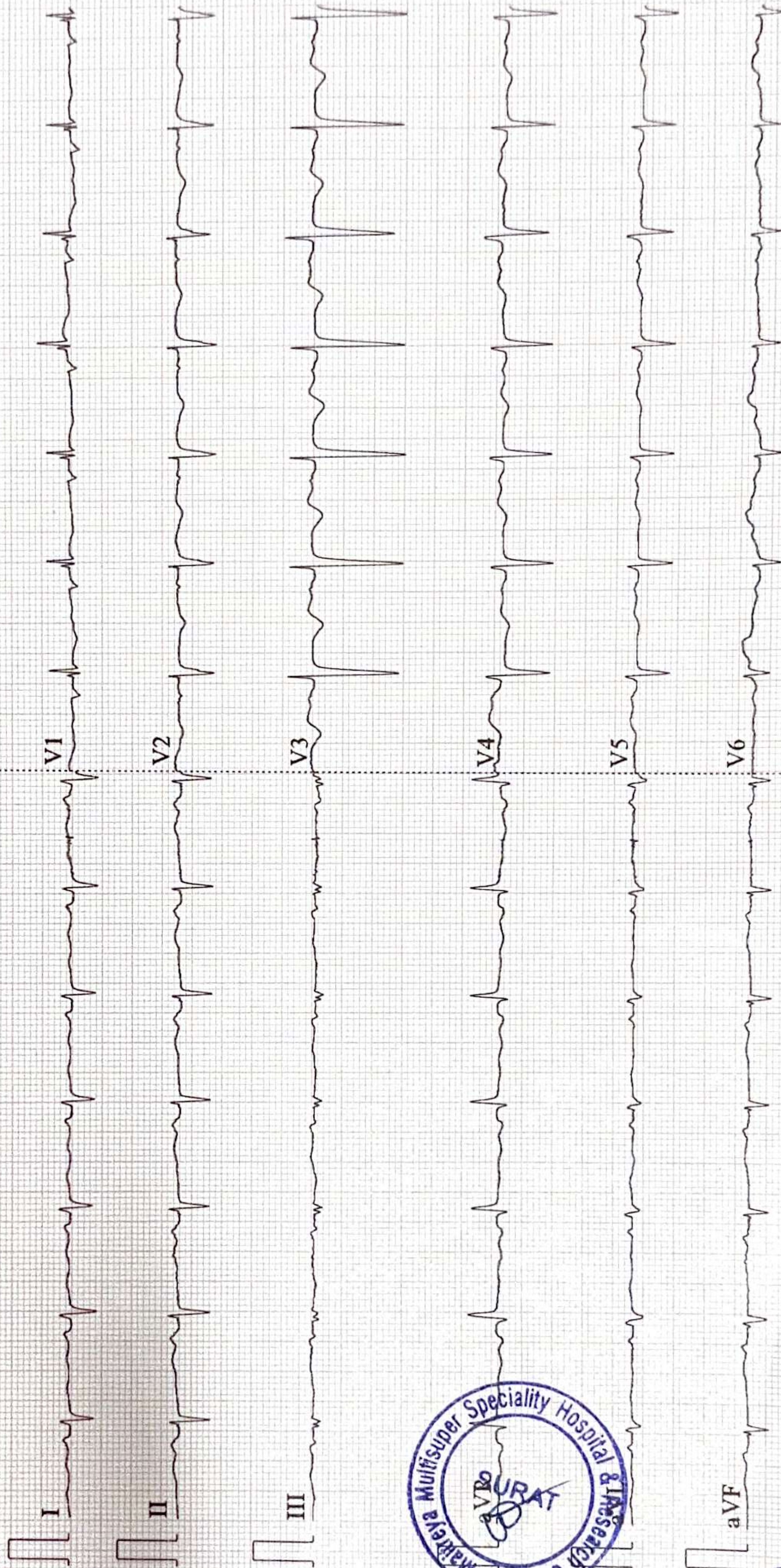
PR : 165 ms

QRS : 83 ms

QT/QTc : 440/520 ms

P/QRST : 30/230/55 °

RV5/SV1 : 0.188/0.105 mV



ID: 415

Male Years

Req. No. :

SAROJ - AGARWAL
UHID: 59568 Age: 63 Yrs./Female
Indoor No: 1/0726/907
DOA: 05-Jul-2026 9:22 pm
DR. SANJAY AGRAWAL
PPT DR SANJAY AGRAWAL

07-07-2026 07:42:45 AM

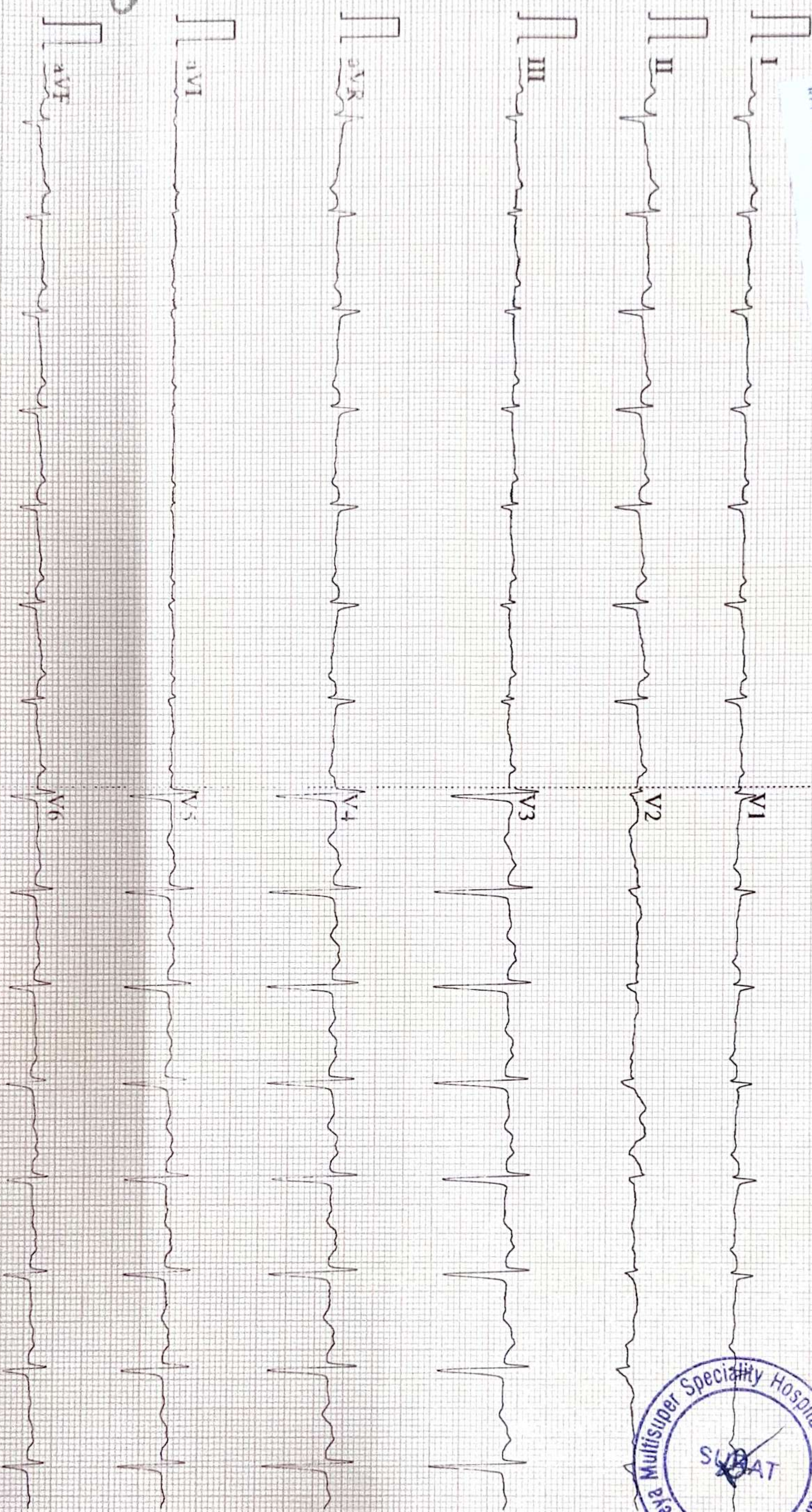
HR	: 90	bpm
P	: 105	ms
PR	: 181	ms
QRS	: 86	ms
QT/QTcBz	: 377/463	ms
P/QRS/T	: 62/223/59	°
RV5/SV1	: 0.420/0.000	mV

Diagnosis Information:

Sinus Rhythm
Abnormal q Wave(V1)
Right Ventricular Hypertrophy
Inverted T Wave(V3,V4)
Indeterminate Axis

Report Confirmed by:

Dr. Vidwan Reddy
7-4-2026



25 Hz AC 50 25 mm/s 10 mm/mV V2.21 SEMIP 1.1.1.1 Serial V6

Male
Years
Req. No. :

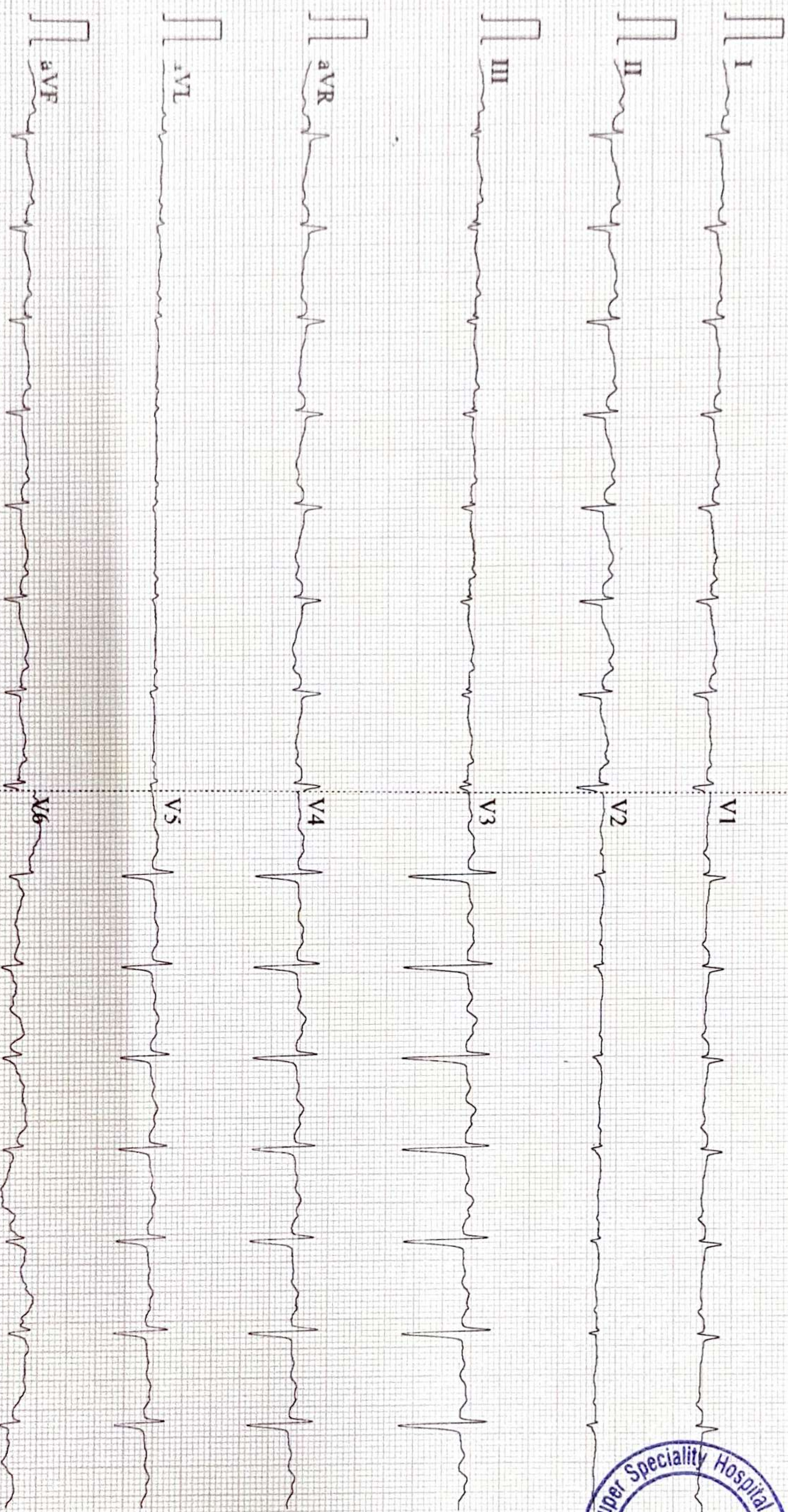
08-07-2026 08:33:49 AM

HR : 94 bpm
P : 91 ms
PR : 158 ms
QRS : 91 ms
QT/QTcBz : 422/529 ms
P/QRS/T : 62/241/56 °
RV5/SV1 : 0.344/0.000 mV

Diagnosis Information:

Sinus Rhythm
Incomplete Right Bundle Branch Block
Abnormal q Wave(V1)
Right Ventricular Hypertrophy
Prolonged QT Interval
Indeterminate Axis
Report Confirmed by:

SAROJ - AGARWAL
UHID:59568 Age: 63 Yrs./fema
Indoor No: I/0726/907
DOA:05-Jul-2026 9:12 pm
DR. SANJAY AGRAWAL
REF.:DR. SANJAY AGRAWAL



DIGITAL X RAY
4D SONOGRAPHY PREGNANCY
SONOMAMMOGRAPHY
COLOR DOPPLER



3D WHOLE BODY CT SCAN
1.5T ADVANCED MRI
CT SC
BIO
RAPHY
DURES



Scan to Book Appointment

Patient Name : SAROJ AGARWAL	Age/Sex : 63 Y / FEMALE
Ref. By : DR. SANJAY AGRAWAL	Date : 05/07/2026

MULTISLICE HRCT SCAN OF CHEST (PLAIN)

Multislice axial CT sections of Chest were taken at 1 mm interval using high spatial frequency algorithm.

CLINICAL INFO: Patient is a follow-up case of CA endometrium. Previous PET CT scan dated 22 June 2026 is available for comparison.

FINDINGS

Moderate bilateral pleural effusions are noted (left > right), with left sided fissural effusion. As compared to the prior PET/CT, there is increase in the volume of the pleural effusions.

Approximately volume of the pleural effusion on right side measures 150–180 cc on the right side and 180–200 cc on the left side.

Chronic subsegmental atelectasis is seen involving the left lower lobe and lingular segment of the left upper lobe.

Mild bilateral pleural thickening is noted - similar as compared to the prior PET/CT.

No evidence of any suspicious lung nodule is seen.

A few mildly enlarged prevascular lymph nodes are noted, largest measuring 8 x 7 mm. A few small lymph nodes are seen in the right upper paratracheal and pretracheal region, largest measuring 7 x 6 mm - similar as compared to the prior PET/CT.

A few mildly enlarged bilateral axillary lymph nodes noted with presence of fatty hila.

No evidence of any pericardial effusion.

The pulmonary artery is mildly prominent and measures approximately 3.4 cm in diameter. Ascending aorta measures approximately 3.2 cm in diameter.

Bilateral lung parenchyma shows no evidence of consolidations.

No obvious bronchiectasis or honeycombing is seen.

Both sided lung parenchyma shows no evidence of cavitation.

Trachea and main bronchi show no gross abnormality.

Cardiac size is within normal limit.

No evidence of any aggressive osteolytic lesion is seen in the visualized dorsal spine.

Dr. Jenish Shah
M.D. Radiodiagnosis

Dr. Parth Bhut
DNB Radiodiagnosis

Dr. Deepa Lalwala Gandhi
DMRD, DNB Radiodiagnosis

Dr. Ankit Chauhan
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NAV KAR ALTHAN BRANCH

X-RAY USG

217, 3rd floor, Atlanta Shopping Mall,
VIP Road-Althan Char Rasta, Surat - 395017.

NAV KAR VESU BRANCH

CT - MRI - USG - X-RAY

204-207-208, 2nd floor, Western Vesu Point,
Vesu Char Rasta, Surat - 395007.



NAV KAR AI CHATBOT -

CT SCAN / MRI - 6359022333
AMBULANCE - 6359622333

6359422333

ALTHAN BRANCH - 7485922333

VESU BRANCH - 9512822333

DIGITAL X RAY
4D SONOGRAPHY PREGNANCY
SONOMAMMOGRAPHY
COLOR DOPPLER



3D WHOLE BODY CT SCAN
1.5T ADVANCED MRI
CT SC
BIO
RAPHY
DURES



Scan to Book
Appointment

Patient Name : SAROJ AGARWAL	Age/Sex : 63 Y / FEMALE
Ref. By : DR. SANJAY AGRAWAL	Date : 05/07/2026

IMPRESSION

- Moderate bilateral pleural effusions (left > right), with left sided fissural effusion - As compared to the prior PET/CT, there is increase in the volume of the pleural effusions.
- No evidence of any acute infectious pneumonia.
- Chronic subsegmental involving the left lower lobe and lingular segment of the left upper lobe.
- Mild bilateral pleural thickening - similar as compared to the prior PET/CT.
- Mildly enlarged prevascular, right upper paratracheal and pretracheal lymph nodes - similar as compared to the prior PET/CT.
- The pulmonary artery is mildly prominent - measures approximately 3.4 cm in diameter. No significant cardiomegaly.

Thanks for the referral.

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